



CEI RESEARCH: Ambulatory Strategy in Mexico

Business Development Department
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A strategy to move beyond CHRISTUS Muguerza's current CAM footprint and build a purpose-built, digitally enabled, payor-connected ambulatory platform.



Contents

1. Sources of Information
2. Executive Summary
3. Competitive Benchmark: What's Done the Right Way
4. Opportunity: Market Logic, Current State & Strategic Rationale
5. The CEI/Muguerza Ambulatory Model: Strategy & Proposal
6. Technical Analysis and Digital Implementation Approach
7. Ambulatory Platform: Implementation Roadmap
8. Conclusion: What we take away and why its Market-Proven
9. Appendix



1. SOURCES OF INFORMATION

All the information related to Monterrey's macroeconomic profile, demographics, history, as well as the assumptions and comments presented in this document, is derived from various sources, including interviews, publications, reports, and journal articles gathered in our previous business cases analysis from Alivia Clínicas Especializadas, Oncare, Salud Digna and USPI. All sources are listed in this section:

1. Alivia Business Case Study – [Alivia Disruptive Model](#)
2. Oncare Biasness Case Study – [Oncare Healthcare integrated Model](#)
3. Salud Digna Business Case Study – [The McDonalds of Health](#)
4. USPI, Tenet Ambulatory Strategy – [USPI](#)
5. Healthcare Experiences Grow Patient Loyalty – [Accenture](#)
6. Ambulatory Strategy Presentation - [PPTX](#)



2. EXECUTIVE SUMMARY

This document provides the culmination of 4 business case studies developed by the Business Development department at CHRISTUS CEI. It consolidates and provides an in-depth analysis of: Alivia Clinicas Especializadas, Oncare, Salud Digna and Tenet's USPI Ambulatory division. All four pioneers of disruptive value propositions that have changed the ambulatory healthcare market, their models have pinpointed clear areas of opportunities for Muguerza's system which could represent an increase in Lives Served and hence, an expansion in our mission.

Each model represents a distinct approach to a shared structural problem: the gap between the current needs of patients covered by Private Healthcare Insurance (PHI) and what traditional hospital systems provide. USPI tenet Healthcare ambulatory surgery platform and the largest of its kind in the U.S. proves the model effectiveness at scale by achieving a physician-aligned, payor-connected, single-specialty operating model that consistently delivers hospital-grade outcomes outside the hospital. The other three show how each dimension of the model can be tropicalized in Mexico. ALIVIA has built a premium, insurance-integrated infusions platform with a strong physician referral engine and white-glove patient experience. ONCARE has scaled a tech-enabled ambulatory model with a physician-facing app that drives referrals beyond financial incentives, enhancing patients' treatment for ambulatory surgeries, lab and imaging centers and oncology services, all within one single ecosystem. SALUD DIGNA has demonstrated that Lean Manufacturing principles applied to healthcare can generate high-volume, low-cost diagnostics at scale, without sacrificing patient throughput or quality perception.

Executive Snapshot: What has the competition accomplished?

Category	Alivia Clínicas	Oncare	Salud Digna	USPI
Model	Specialty ambulatory infusion & pharmacy platform.	Integrated specialty care network + ambulatory surgery.	High-volume diagnostics, lab, imaging & optics.	Physician-syndicated ASC platform; site-of-care cost arbitrage.
Active Clinics	24 clinics	6 specialized centers + 1 ambulatory hospital	235 clinics	533 ASCs + 26 surgical hospitals
Geographic Footprint	15 states	Monterrey & Mexico City	32 states, 78 cities	37 U.S. states
Patient Volume	25,000+ patients / 200,000+ infusions since inception	45,000 Patients served	18M patients / 28M services (2024)	~2M procedures / year
Est. Annual Revenue	~USD \$55M (2023)	Not disclosed	~USD \$420M (2024E)	~USD \$5.17B (2025); 39.2% adj. EBITDA margin
Target Segment	AB / C+ with private insurance	AB / C+ with private insurance	C- to C / B price-sensitive	Commercially insured outpatients (U.S. payer mix)
Key Alliances & Partnerships	35+ insurers, 24 pharma partners, Farmacias Benavides ecosystem	15+ insurers and 100+ specialist physicians	IMSS/ISSSTE agreements, lease model, optics supply chain partnerships	11,000+ affiliated physicians; Tenet ownership; surgeon JVs; health-system co-ownership

When we combine these four models, we shed light on a major opportunity for CHRISTUS Muguerza. The opportunity is not to build ambulatory care from scratch, nor to suggest that Muguerza lacks ambulatory assets. The system already has the hardest pieces in place: existing clinics and CAMs, hospital infrastructure, strong brand equity, physician relationships, and long-standing insurer connectivity. The real opportunity is to transform these existing assets into a new competitive, standardized, digitally enabled ambulatory platform – one capable of capturing the outpatient volume that is currently leaking to specialized competitors such as Alivia, Oncare and other competitors.



3. COMPETITIVE BENCHMARK: WHAT'S DONE THE RIGHT WAY

3.1 Alivia Clinics: Ambulatory Infusion Therapies

Alivia is a multi-site outpatient specialty care platform that was built around an existing structural gap between the public healthcare system and the private healthcare dimension, in which patients with complex recurring conditions: oncology, rheumatology, MS, Crohn's, nephrology – that require high-cost infusion therapies but do not need full hospitalization. Rather than competing as a general clinic, Alivia positioned itself at the intersection of the infusion treatment patient's journey.

Key insights: What do they do right

1. **Payor integration as a competitive advantage:** With more than 35 insurer relationships and direct payment and billing integration, Alivia removes the biggest friction point in the private specialty care: administrative coordination with Private Healthcare Insurance (PHI) payors.
2. **Physician referral network via Soy Doctor and medication through Alivia RX:** Alivia has formalized its physician relationship model into a structured program that partners with 1,500+ specialists. Physicians refer patients, Alivia handles scheduling, medication coordination via their specialty pharmacy in partnership with Farmacias Benavides (Alivia RX), insurance pre-authorization even with direct payment, and administration. Physicians retain clinical authority. One referral generates a recurring patient relationship.
3. **Price advantage over hospitals:** Alivia self-stated that their ambulatory model is estimated to be up to 50% more affordable than hospital-based administration for the exact same treatment. Chemotherapy administration alone benchmarks at MXN \$3,150 at Alivia vs. MXN \$7,500 at Muguerza, a 2.4x gap that is compelling for both patients and payors.
4. **WhatsApp – automated patient coordination:** Their entire patient intake is through their automated WhatsApp, including scheduling, location assignment and insurance paperwork, direct quotation is made through this channel, reducing friction and enabling fast response without complex digital infrastructure.
5. **Vertical integration across the full treatment journey:** Alivia is not just an infusion site, it combines outpatient administration, specialty drug access (Alivia RX), and insurance management under one roof. This allows the company to capture margin at multiple points of the treatment cycle, not just at the clinical visit.
6. **Modernism & comfort: a complete infrastructure:** Alivia clinics are specifically designed to not feel like a hospital. Reclinable chairs, private infusion rooms, curated aesthetics, natural light, and a calm environment are core to the patient's experience, delivering an experience through **Dignity** and **Compassion**.

Main takeaway for our ambulatory strategy:

The real opportunity in the ambulatory infusion therapies market is not the clinics themselves. These are the initial catch, but the retention factor is the coordination layer, **whoever controls the insurer relationship, the physician referral flow, clinical records consolidation and the medication pathway controls the patient**. Muguerza already has the brand and is a well-known name for the insurer companies. The missing piece is the **direct payment & alliances**, and the strengthening of the **physician referral benefits** (not financial but through digital tools such as Soy Doctor App and insurance paperwork to incentivize direct payments). Finally, the defining factor that we take away from Alivia is their UX, **the modernism and sense of comfort that their clinics offer is a key differentiator**.



3.2 Oncare: The Northern Mexico Integrated Healthcare model

Oncare is a complete healthcare system that captures and exploits the entire patient journey even before it starts, from detection to treatment and follow-up until completion and the option to return at any moment without any friction. Oncare connects all of them under a single coordinated system that englobes several specialties and offers treatment to all of them from preventive check-ups and early detection to ambulatory infusions, hemodialysis, endoscopy, and minimally invasive surgery. With 6 specialized centers, 100+ affiliated physicians across 15+ specialties, and 45,000+ patients served.

Key insights: What do they do right

1. **Full patient journey integration:** Oncare is the only private outpatient platform in Nuevo León that combines early detection and ambulatory treatment across oncology, hematology, renal care and pulmonology, and a short stay surgical hospital. All operating as a single coordinated system. Every stage feeds the next, eliminating the administrative and logistical fragmentation tax that patients, physicians, and insurers absorb in traditional hospital-centric models.
2. **Open physician network:** Besides their salaried staff that manage their specialized centers, Oncare structured itself as a healthcare operating system for independent specialists. Over 100 physicians use Oncare's infrastructure insurance integration, and digital platforms to treat their patients. The value proposition for physicians is not primarily economical, it relies on the operational advantages that Oncare provides, high-end diagnostics and treatment technology without individual CAPEX expenses, administrative concierge for insurance paperwork, faster payment collections, and a digital platform to manage their patients in real time.
3. **Dual digital Infrastructure:** Oncare has a key differentiator – their customer relationship channel is not a single app, is a dual platform shaped by their medical app "[Oncare medical record](#)" a physician faced administrative channel where doctors can manage their patients, plus their "[Oncare Patient Portal](#)", a centralized platform for patients with all their clinical information and documentation. These two platforms create a closed-loop ecosystem where clinical data, administrative workflows, and insurance tracking join into a single experience for both doctors and patients.
4. **Payor connected access:** Direct alliances with a variety of top insurance companies like GNP, MAPFRE and MetLife allow patients to access high-specialty services without initial out-of-pocket expenses or reimbursements. By managing preauthorization and insurance paperwork, Oncare reduces the friction that often happens between the patient, the insurer and the physician.
5. **Multi-channel branding:** Oncare has maximized their current infrastructure, they have evolved beyond its core oncology brand by hosting independent specialty groups – The Gastro center and Pulmonary Center – Under different brands that share Oncare facilities. These fill their inactive capacity in surgical suites and recovery rooms while allowing these other targeted patients to get the same quality service, without the psychological barrier of a dedicated cancer center.

Main takeaway for our ambulatory strategy:

Physician loyalty is not bought, it is achieved through infrastructure access, digital tools and administrative enablement. Oncare shows that their physician app and open staff models are effective mechanisms that convert independent specialists into a captive referral network. Muguerza already has clinical credibility and specialist relationships. **The missing piece is a structured digital layer**, a physician-facing platform that makes Muguerza's ambulatory infrastructure the easiest, most



administratively seamless environment for a specialist, **we need to achieve clear incentives that allow us to gain loyalty** with our own tenants.

3.3 Salud Digna: Operational Excellence: “The McDonalds of healthcare”

Salud Digna is the largest private diagnostics network in Mexico; they developed a structurally robust healthcare business model that has been regarded as one of the best healthcare business models in Latin America and has been a topic of research by Harvard Business School. Founded in 2003 in Sinaloa by Jesus Viscarra, a former industrial executive that built Salud Digna under a single conviction: The constraints limiting medical diagnostics in Mexico were not medical – they were operational.

Key insights: What do they do right

1. **Lean manufacturing applied to healthcare:** Every Salud Digna clinic runs like a factory: Standardized protocols, parallel task execution, maximized patient flow, and real time performance monitoring. While a physician examines one patient the other one is being prepared. Every process is documented into checklists. Every clinic manager reviews four operational quadrants daily via a Personal Digital Assistant. This is not efficiency as a goal; it is efficiency as the operating model.
2. **Hub-and-spoke infrastructure:** MRI and CTE scanners sit in their Sinaloa and Mexico City Hubs, while lighter spoke clinics generate patient volume and feed them into high capex nodes, maximizing asset utilization and eliminating any double investing.
3. **Leased assets & equipment strategy – low CAPEX:** A key characteristic of Salud Digna’s model is that they don’t buy their equipment, they have a usage-based leasing agreements with Roche and GE Healthcare, amongst other providers. Also, Roche donates (Salud Digna is a non-for profit organization) or sells their equipment at deep discounts in exchange for exclusive consumable supply chain contracts. Salud Digna is Roche's 6th largest client worldwide. GE HealthCare manages 140+ imaging devices across the network under a full lifecycle agreement. New clinics open faster, balance sheets stay lean, and expansion risk is shared with suppliers.
4. **Eyeglasses as their margin generator:** With approximately 50% gross margins, the optics business model is their financial engine that funds the heavy equipment like the MRI machines. The glasses are their jewel hiding in plain sight, they absorb lower margin diagnostics services and make the entire pricing model viable.
5. **WhatsApp as the primary UX:** 91% of all patient interactions, booking, payment, results delivery and appointment reminders, happen inside WhatsApp. No app download required. No call center dependency. In a country where low-income users have unlimited WhatsApp but limited mobile data, this is the front door to the entire operation.

Main takeaway for our ambulatory strategy

Salud Digna’s operating and economical model makes them the leading diagnostics service in Mexico. Their lean manufacturing philosophy and their hub-and-spoke network, and asset-light CAPEX structure are what make pricing, scale, and speed of expansion possible. For CHRISTUS, the direct application is not to replicate Salud Digna's diagnostic model – **It is to borrow its infrastructure logic: standardize ambulatory clinic operations**, deploy new ambulatory clinics near Muguerza hospitals, **combined with a replication of their complete all around WhatsApp model.**



3.4 USPI/ TENET: Healthcare Ambulatory Strategy

United Surgical Partners International (USPI) is Tenet Healthcare's ambulatory surgery platform and the most proven ASC operating model in the United States. It was built on an industrial conviction (similar to Salud Digna), not a clinical one: most surgery does not need the fixed-cost apparatus of a full hospital, so moving eligible procedures into focused, standardized, physician-owned facilities delivers the same outcome at a fraction of the cost. ([USPI-Business case study](#))

Key insights: What do they do right:

1. **Site-of-care arbitrage is the entire thesis:** The same procedure costs payers 40–60% less in a freestanding ASC than in a hospital outpatient department (colonoscopy ~38% cheaper, cataract ~44%, outpatient joints ~40%). USPI exists to capture that spread.
2. **Two-level architecture:** platform vs. facility. Tenet owns 100% of USPI at the corporate level. The "joint ventures" live only at the facility level – each center is a separate legal entity where the platform holds a controlling stake (~51%+) and local surgeons buy minority equity (20–49% total, 1–10% each). USPI is the brain (playbook, GPO, payer contracting, revenue cycle); the centers are the body.
3. **Physician syndication as the behavioral engine:** Surgeons buy equity at fair market value; distributions are strictly pro-rata to ownership, never tied to referral volume – the bright line that keeps the model clean. The result: owners behave like owners. They start on time, standardize supplies, compress OR turnover, and bring their cases to *their* center because every case raises the value of their stake.
4. **Three stacked revenue streams:** (a) Facility distributions – quarterly free cash flow pro-rata to ownership; (b) Management fees, 3–6% of *net facility revenue*, earned on 100% of revenue including the physicians' share; (c) Multiple arbitrage – single ASCs trade at 5–8x EBITDA, scaled platforms at 11–17x. Every center added to a standardized platform is worth more inside it than standalone. This is the core long-term value-creation mechanism – and the reason to build a *platform*, not isolated centers (VMG Health; Scope Research; Weaver).
5. **Bundled pricing is how the arbitrage is captured:** USPI prices each procedure as one fixed in-network rate per episode of care, not itemized fee-for-service – the only way the site-of-care saving becomes a margin both the operator and the payer can plan around. The mechanism translates directly to Mexico: CEI can offer insurers a GMM fixed-price bundle to contain medical inflation.
6. **A replicable operating playbook:** GPO-driven procurement and implant standardization, block scheduling targeting ~70% OR utilization (the breakeven threshold), monthly contribution-margin-per-case and per-surgeon analytics, centralized revenue cycle and payer contracting, and a dual growth engine (de novo + acquisition with physician re-syndication at close).

Main takeaway for our ambulatory strategy

USPI is the structural skeleton CEI is proposing to tropicalize. Where Alivia, Oncare and Salud Digna each contribute one proven practice, USPI contributes the whole operating architecture: **a platform that owns the playbook, procurement, payer contracting and revenue cycle**, sitting above facilities where surgeons hold real equity. Syndication is the engine, owners behave like owners and bring their cases to a center whose value they share, with distributions tied strictly to ownership and never to referral volume.



4. OPPORTUNITY: MARKET LOGIC, CURRENT STATE & STRATEGIC RATIONALE

The Competitive benchmark presented in the previous section shows a clear conclusion: the ambulatory specialty market in Mexico is being captured by those who solved problems that CHRISTUS Muguerza has historically left unaddressed, which can be attributed to lack of innovation – Nevertheless, Muguerza has all the necessary capabilities to make these changes. We just need the correct action plan and business model to shift from hospital-based inpatient care delivery to a complete combination of both ambulatory service and the existing ones.

CHRISTUS Latam is no stranger to ambulatory business models, UC CHRISTUS has its own fully specialized “UTM’s”; lab & imaging centers specialized in sample collection and ambulatory services, with 73 operational ambulatory units all around the territory, plus an optics department at San Carlos de Apoquindo Hospital. In Mexico, Muguerza has CAM’s: an ambulatory attempt by CHRISTUS, that needs work but serve as an initial step for our fully integrated ambulatory model.

CHRISTUS Muguerza has this clear opportunity to enter the ambulatory market, not by optimizing what it already has, but by implementing an already existing model, that has shown success throughout the Mexican territory. These are the two converging factors that make this proposal a real opportunity:

1. Demand is already there and is currently being captured by others, the specialty outpatient population in Nuevo León and the broader Northern Mexico region is not hypothetical. It exists today. A meaningful portion of it is already being served by standalone ambulatory operators, not because those operators are clinically superior to Muguerza, but because they offer a more convenient, seamless, more affordable, and less administratively burdensome experience for recurring treatment. Every recurring infusion patient treated at a competitors’ facility is a patient relationship – and a medication margin, an insurer touch point, and a referral stream – that does not return to Muguerza.
2. The insurer incentive is structural and growing. Major Private Healthcare Insurance (PHI) payors are under sustained financial pressure from specialty care claims inflation. Ambulatory administration of chemotherapy, biologics, and infusion therapies at a dedicated outpatient facility reduce the total cost of claims significantly versus hospital-based administration. Insurers have a direct financial incentive to steer patients toward certified ambulatory providers. Muguerza's existing payor relationships – already active, already trusted – are the fastest path to becoming the preferred ambulatory destination for insured specialty patients in the region.



4.1 Strategic Rationale & Muguerza differentiators

CHRISTUS Muguerza enters this space with a set of clear structural advantages that none of the competitors here analyzed were able to start with. We aren't reinventing the wheel; we already have a wide starting ground that we can exploit.

What CHRISTUS Muguerza has that no ambulatory operator can replicate:

- **Brand equity and clinical credibility:** Muguerza is the most recognized private hospital brand in Northern Mexico, dominating the market in one of the most affluent corridors of the nation, is already trusted by patients and has built relationships with physician and insurers for decades.
- **Physical infrastructure – A fully operational hospital adjacent to ambulatory units:** Patients who require escalation beyond ambulatory care can be moved upstairs. No transfer, no new intake, no lost continuity. This is a clinical safety net that Alivia, Oncare, and every standalone ambulatory operator simply cannot offer.
- **Existing and growing Private Healthcare Insurance (PHI) payor connectivity:** Muguerza's years of active insurer relationships represent a pipeline that took Alivia a decade to build with 35+ payors. The ambulatory channel does not need to earn insurer trust from zero – It needs to extend and formalize existing relationships into a new, lower-cost care setting. Plus, our current projects with Insurance Brokers such as Sekura (Mexico's largest broker), position ourselves in the best path to develop specific products and strategies that increase efficiencies with patient-payor operations.

4.2 The current Hospital Experience & CHRISTUS Muguerza Current Ambulatory Platform

CEI'S goal is to redefine the current ambulatory attempt that Muguerza has created via their ambulatory clinics. The situation relies on the fact that the current model contradicts the logic of an ambulatory service. For instance, Muguerza's "Ambulatory Clinic" at San Pedro operates as separated entity but feels almost identical to the hospital counterpart, it appears to be a mini hospital instead of an ambulatory clinic.

The feel and aesthetics of the facility are cold and do not create a sense of safety for our patients. This "ambulatory clinic" that Muguerza has created is a vague attempt at competing against models that Muguerza has no idea how they work.

Measured against the USPI logic, this is the core failure. USPI's entire thesis is site-of-care arbitrage – moving eligible procedures out of the hospital's fixed-cost apparatus and into a focused, lower-cost setting that delivers the same clinical outcome. Muguerza's current ambulatory clinic does the opposite: it carries the hospital's cost structure, feel, and operating logic into a setting that was meant to escape them. As a result, it captures none of the arbitrage that makes the ambulatory model economically disruptive in the first place. The building is ambulatory in name; the economics and the experience are still hospital.

Muguerza CAMs are first contact units that offer general consultation, basic imaging and some specialty physician services. They are 8 CAM centers that operate as individual units that provide lower complexity access points outside hospitals. However, they lack the Alivia and Oncare look & feel, location, modernization, comfort and user experience. They function as independent clinics that try to address lower income deciles without the care of creating a complete operational model and a robust business model.



Muguerza does not lack ambulatory infrastructure; it lacks the modernization, plus digital and operational layers required to make that infrastructure behave like an integrated ambulatory platform.

4.2.1 The Current Hospital Experience

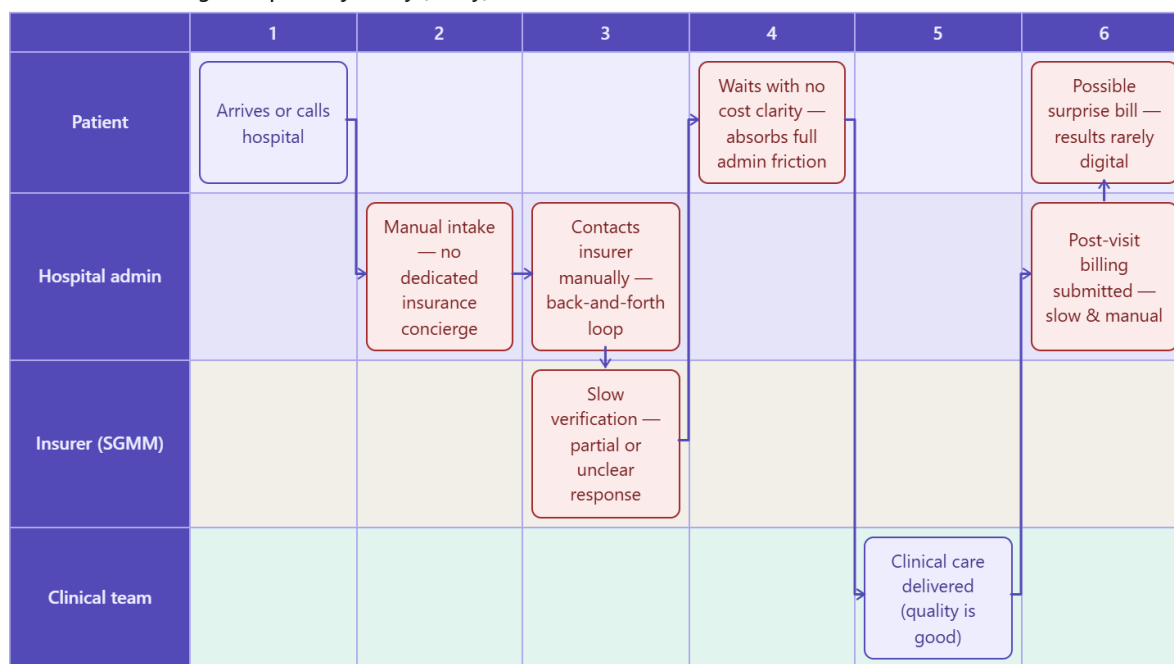
A Muguerza patient today enters a well-equipped, top clinically recognized hospital. The care itself is not the problem. What breaks the experience is everything surrounding it. Muguerza's model is structurally designed for low volume and high price – a model that works for complex inpatient events but creates disproportionate friction for the recurring specialty outpatient who simply needs to show up, get treated, and leave.

The most consistent pain point is insurance coordination. When a patient presents Private Healthcare Insurance coverage, the process of validating the policy, confirming coverage scope, obtaining pre-authorization, and coordinating billing is slow, opaque, and largely absorbed by the patient. There is no dedicated concierge function – no specialized team whose sole job is to own that interaction with the insurer on the patient's behalf. The patient waits. The process is unclear. And in many cases, the out-of-pocket exposure at the point of care is uncertain until well after the clinical event.

Note: This insight comes from Muguerza hospitals here in Monterrey, we are not taking into accounts those who have a specialized team for insurance administration in other Muguerza hospitals outside the state.

Current journey

Current state — Muguerza patient journey (today)



□ Normal step □ Friction / pain point



5. THE CEI/ MUGUERZA AMBULATORY PLATFORM: STRATEGY & PROPOSAL

Once we established the current ambulatory state for Muguerza, the Business development team started with the process of requirement recollection needed to achieve a successful **Muguerza Ambulatory model**.

5.1 CEI Ambulatory Premises: What Are We Aiming For?

The ambulatory platform CEI proposes for Muguerza is a purpose-built, hospital-adjacent, digitally integrated, payor-connected specialty care network. Its operating architecture is built on the USPI/Tenet model the most proven ambulatory platform in the world and tropicalized to the Mexican market by adapting the specific practices that made Alivia, Oncare and Salud Digna the current leaders in ambulatory service clinics in northern Mexico: Alivia's WhatsApp-integrated patient journey and Salud Digna's operational efficiency. USPI provides the skeleton; the local leaders provide the practices that already work in our market.

It is designed to serve the AB/C+ private healthcare insurance patient who requires recurring specialty treatment, infusions, diagnostics, lab, imaging and ambulatory surgery, but does not need full hospitalization. Each site operates as a single-specialty, standardized clinic rather than a broad hospital department: that focus is what makes processes repeatable, costs predictable, and bundled pricing with insurers viable. The model looks, feels, and functions like a premium specialty clinic sited near Muguerza's full hospital infrastructure for clinical escalation. The core objective of the model is to build the proprietary digital infrastructure required to eliminate the administrative friction that has historically driven recurring patients toward competitors, the full value proposition is explored in detail in the following sections.

This is not a new service line added to the hospital. It is a separate operating model – built on the USPI operating backbone – with its own patient experience, its own physician engagement logic, its own insurer coordination infrastructure, and its own economics, operational discipline and digital infrastructure.

Why does this proposal move beyond CAMs?

The CAMs were not designed to compete against Oncare and Alivia. They function as general first contact units: lower complexity, lower acuity and structurally disconnected from the insurer integration and physician referral logic that drives recurring specialty volume. Three factors make building a new model the right decision to retrofit the existing one:

1. **Location:** CAMs are not positioned in the high-income corridors of the city, where our AB/C+ Private Healthcare Insurance target patients live and seek care. Relocating or rebuilding them defeats the asset-light logic.
2. **Investment:** The investment needed to transform these CAMs from generic cold ambulatory clinics to Alivia and Oncare level clinics is far beyond the initial investment needed for new ambulatory clinics inside Muguerza hospitals infrastructure, or having new ones through a lease model.



5.2 Growth Model – Organic and Inorganic

The business development team offers two possible growth models; Muguerza can decide which of the two applies to their needs.

Model 1 – Organic Growth: Activation of existing space at Muguerza facilities as dedicated ambulatory zones. This model prioritizes speed, capital efficiency, and immediate leverage of existing infrastructure, brand, and insurer relationships. The ambulatory unit operates as a clinically and experientially distinct environment within the Muguerza hospitals – designed and operated as a specialty clinic, not a hospital department – with direct escalation access to full hospital capabilities when clinically required.

Model 2 – Inorganic Expansion Asset-light spoke deployment in Monterrey, anchored on existing Muguerza hospitals as clinical hubs. Unlike the organic model – which expands inside Muguerza's current hospital footprint – this model stands up new ambulatory spokes in fully leased locations *near, but outside*, those hospitals: space, equipment, and technology all leased rather than built. This keeps CAPEX low and preserves the option to exit any site that underperforms, with no permanent construction commitment. The result is faster time-to-market and materially lower financial risk than greenfield development, while keeping every spoke within clinical-escalation distance of a Muguerza hub.

Monterrey HUB & SPOKE Opportunity: The business development team has identified three move-in-ready leased properties, each suitable for fit-out as an ambulatory clinic, the locations are near Muguerza hospitals in San Pedro Garza Garcia.

Every site can function as their own independent service line but can handle escalation duty if necessary, making referral easier for patients and physicians. ([Lamundi](#)) ([Imuebles24](#)) ([Propiedades.com](#))

Six key design principles:

These principles define what the CEI ambulatory model must be – and equally, what it must never become.

- 1. Clinic feel, not hospital feel:** Every physical and experiential element of the ambulatory unit must signal that this is not a hospital. Curated aesthetics, natural light, reclining infusion chairs, private treatment spaces, and a calm, unhurried environment are not amenities – they are core to the model. The moment a patient feels like they are inside a hospital, the value proposition begins to erode.
- 2. First operations** pre-authorization is completed before the patient arrives. A dedicated administrative team – whose sole function is insurer coordination – owns the entire relationship with the Private Healthcare Insurance payor on the patient's behalf. The patient's role is to show up. Every administrative friction point that currently falls on the patient is absorbed by the ambulatory operation.
- 3. Physician as the primary referral engine** Patient acquisition in the ambulatory model does not come from marketing. It comes from physicians. The model is designed to make referring to Muguerza's ambulatory platform the easiest, lowest friction, most administratively advantaged option for the independent specialist. The physician refers. Muguerza's ambulatory team handles scheduling, pre-authorization, treatment coordination, and outcome reporting. The physician retains clinical authority and gains operational simplicity.
- 4. Hub-and-spoke on existing Muguerza infrastructure** Capital efficiency is structural, not optional. Every spoke clinic is deployed on existing Muguerza real estate or under a full-lease model – space, equipment, and technology – with Hospital Muguerza serving as the clinical hub and escalation net.



No greenfield hospital builds. No duplicated fixed costs. Expansion speed is a direct function of asset-light execution.

5. **Muguerza Connect as the operational backbone** Every patient interaction – booking, pre-authorization status, results delivery, appointment reminders, follow-up – flows through a single digital channel. Muguerza Connect, already in active beta, is the platform that connects the patient, the physician, the administrative team, and the insurer into one coordinated operating system. Friction disappears because the platform is designed to absorb it.
6. **Operational Efficiency and Service Line specialization:** Each ambulatory spoke is built around a single service line rather than a broad clinical mix. That specialization is the key enabler: a narrow, focused scope makes processes easy to standardize and repeat, which lowers per-case costs and keeps them predictable. Predictable, well-defined costs are what make bundled, profitable pricing with insurers viable – adopting the operating logic USPI has proven at scale.

Disruptive Value Proposition: USPI's Business Model, Built on What Already Works in Mexico

This is not a new model invented from scratch it is a synthesis. USPI provides the business model: a specialized, asset-light, physician-syndicated platform that captures the margin a full-service hospital cannot. The Mexican competitors provide the design – the components already proven in the Mexican market – that we layer onto that skeleton. USPI is how the model makes money; Alivia, Oncare and Salud Digna are how it looks, feels and runs on the ground. The result is an adaptation that no single competitor has assembled. The most proven ambulatory business model in the world, executed with the practices already winning in northern Mexico.

Where the USPI business model captures margin. Taking USPI as our skeleton, the model shows that a specialized, asset-light ambulatory platform captures the margin a full-service hospital cannot, because it sheds the fixed-cost apparatus – 24/7 emergency standby, inpatient floors, heavy support staff – that the procedure itself does not require. The same case delivered in a focused outpatient setting costs 40–55% less than in a hospital outpatient department: hospital outpatient prices run 54.9% higher than ASCs for colonoscopy and 44.0% higher for cataract removal ([AJMC](#)), and Medicare pays \$1,671 for a hospital cataract procedure versus \$964 at an ASC ([ASCA](#)). That differential is the margin pool.

Where the margin actually comes from four stacked sources:

- Cost-of-site differential. The ambulatory delivery cost is structurally lower; that gap is captured, not given away, under a negotiated *bundled* ("episode of care") contract rather than itemized billing. CEI inference: in Mexico this translates into a GMM bundled contract with private insurers (sekura), positioning Muguerza as the partner that helps payors contain medical inflation.
- Asset-light footprint. Leased space and equipment instead of greenfield build keeps capital employed low. Industry estimates put a single-specialty, 2-OR center at roughly US\$2–3M of build-out capital with payback near 16 months at strong volume (*industry estimate; to be validated against internal figures*).
- Operational standardization. A narrow specialty scope makes processes repeatable, compresses per-case cost, and keeps it predictable. Breakeven sits at roughly 70% utilization the discipline target that protects margin.
- Multiple arbitrages at scale. A standalone center trades around 7–8x EBITDA ([VGM Health](#)), while regional and national operators managing multiple centers command 11–17x EBITDA ([Scope](#)



[Research](#)). Every spoke added to one standardized playbook is worth more inside the platform than alone.

This is proven, not projected. USPI, Tenet primary earnings engine and the most public reference available, has held an Adjusted EBITDA margin near 40% across 2019–2025, through a pandemic and an inflation cycle, while growing ambulatory revenue to US\$5,172M with EBITDA of US\$2,026M in 2025 ([Tenet FY2025, SEC](#)). Over that period USPI delivered a 15.7% net revenue CAGR and a 14.6% Adjusted EBITDA CAGR, holding a 40.5% segment margin in Q4 2025 ([Tenet Q4 2025 presentation](#)). The margin profile barely moved – that stability, not the headline growth, is the evidence the economics are real at scale.

ASC unit economics	Figure	Basis
EBITDA margin (proven, USPI)	~40%	Verified (Tenet / SEC)
M&A multiple: standalone → platform	7–8× → 11–17× EBITDA	Verified (VMG / Scope)
Utilization breakeven	~70% of OR time	Industry practice
De novo capex (2-OR single-specialty)	~US\$2–3M	Industry estimate
Payback (strong volume)	~16 months	Industry estimate

Three proven local components give the model its substance:

- **Alivia – the patient experience design:** premium, deliberately non-hospital clinic environment paired with a WhatsApp-native journey – Intake, scheduling, quoting, follow-up and insurer paperwork in one channel – that strips administrative friction out of recurring care. This is the look and feel of the model.
- **Oncare – the platform:** Oncare's dual physician/patient digital ecosystem is the direct benchmark for Muguerza Connect: a closed-loop layer that turns independent specialists into a captive referral network and keeps patients embedded. Muguerza Connect is the proprietary version of this – physician, administrative and patient portals consolidated into one owned platform rather than a rented third-party stack.
- **Salud Digna – operational efficiency and immediate results:** Lean, standardized, real-time clinic operations: dashboards tracking volume, queues, turnaround and equipment uptime turn standardization into enforced cost discipline, with managers correcting deviations before they compress margin. This is the same operating logic USPI runs through its contribution-margin-per-case analytics – proven locally at high volume.

Note: a proven margin structure + asset-light capital + a digital layer that holds operating discipline in real time. That combination is what makes the business both profitable and defensible.



5.3 Muguerza Connect: What is it?

The fifth design principle established in the previous section defines “Muguerza Connect” as the operational backbone of the CEI ambulatory platform. This section explains what that means in practice.

Muguerza Connect is the clinical-operational CRM of our ambulatory platform proposal. It is designed to be the operating layer that connects patients, physicians, administrative teams, and insurers into one coordinated ambulatory platform managed by Muguerza's ambulatory team through the clinics.

The role of **Muguerza Connect** is to absorb the administrative friction that could otherwise weaken the new CEI ambulatory model in the same way it currently weakens the patient experience across hospitals and private medical offices at Muguerza. In today's model, patients are often met with small obstacles that create unnecessary friction. In a market where recurring specialty patients have real alternatives – Alivia, Oncare – small frictions create large leakage. [Accenture](#) found that 78% of patients who switched healthcare providers cited “ease of navigation” factors as a reason for leaving. These factors are visible in Muguerza's current digital and administrative model: scheduling, insurance paperwork, pre-authorization follow-up, document sharing, results tracking, and communication between disconnected parties.

The root cause is the lack of a structured administrative and digital infrastructure around Muguerza's independent physicians. Private tenants currently lack meaningful hospital-provided digital services. The system remains decentralized, manual, and traditional, with coordination still depending heavily on phone calls, manual scheduling, and fragmented communication. As a result, many physicians rely on third-party SaaS platforms, paid out of their own budgets, to manage patient tracking, scheduling, revenue, digital medical records, front-desk coordination, patient communication, and other operational needs.

Muguerza Connect is not being proposed as a day-one fix for the entire existing system. Rather, the current system exposes the problem Muguerza must avoid replicating as we build the new ambulatory strategy. The platform will enable our ambulatory clinics to streamline scheduling, insurance pre-authorization tracking, document intake, treatment coordination, results delivery, appointment reminders, follow-up, direct referral communication within Muguerza infrastructure, and additional features explored in the section “[Muguerza Connect: Technical Analysis.](#)”

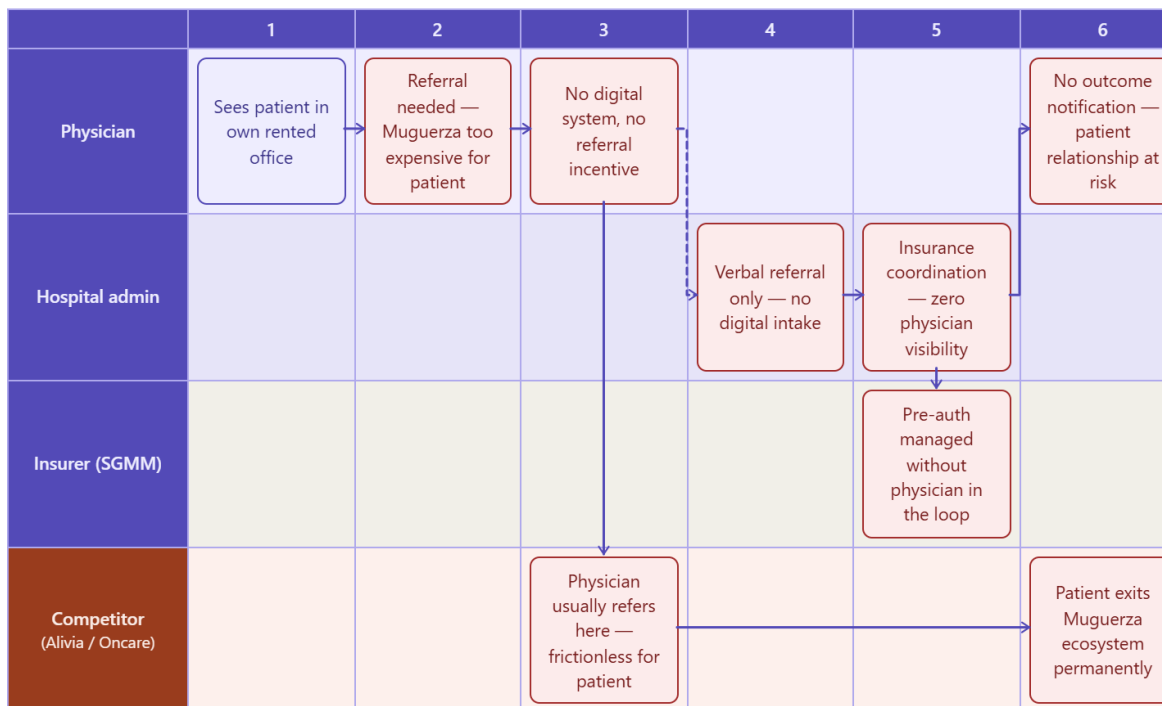
The physician retains clinical authority while gaining operational support. The administrative team gains a structured workflow. The insurer interaction becomes trackable instead of fragmented. In this sense, **Muguerza Connect** is both a CRM and an operational platform. It manages the relationship with the patient, but it also manages the operational journey around that patient. It connects demand generation, physician referral, insurance coordination, clinical delivery, and follow-up into one digital infrastructure.

In the current scope, the platform is intended to be used initially only within the ambulatory clinics, separating the CEI ambulatory model from the private-office model. Over time, **Muguerza Connect** can become a scalable digital service for Muguerza's broader physician network, including private tenants who currently lack hospital-provided administrative technology. [Appendix- Figures](#)



Current physician Journey

Current state — Muguerza physician journey (today)





5.4 Specialized WhatsApp Model for Ambulatory Services serving as initial coordination layer for Muguerza Connect

One of the clearest patterns across the three benchmark models is that the most operationally efficient ambulatory platforms do not compete with clinical quality alone, they compete with excellent customer attention. And in the Mexican healthcare market the customer attention relies on a single communication channel WhatsApp.

Alivia processes its entire patient intake: scheduling, location assignment, insurance quotation, and treatment coordination, through an automated WhatsApp flow. Salud Digna routes 91% of all patient interactions, from appointment booking to results delivery, exclusively through the same platform. Oncare relies on WhatsApp as the primary communication layer between patients, administrative staff, and referring physicians. The common denominator is not the technology itself, but what it solves: the volume problem.

In any ambulatory operation serving hundreds of recurring patients, manual front-desk communication becomes the first point of failure. Staff cannot respond to every inquiry in real time. Patients disengage. Appointments are missing. Referrals leak. The competitors that solved this problem did not hire more administrative staff – they automated the front door.

Muguerza Concierge is the CEI proposal for Muguerza's WhatsApp-native communication layer. It is not a standalone chatbot. It is directly connected to **Muguerza Connect** and serves as the first intake layer for our ambulatory patients. This Chatbot can Schedule, answer frequent questions, indicate current locations and prices, see doctor availabilities, without the need of initial front desk interaction.

The model serves:

- Institutional ambulatory patients accessing the full ambulatory service offering: infusions, laboratory, imaging, specialty consultations, and surgical coordination.

The architecture is intentionally simple. No app download. No registration wall. The patient opens WhatsApp, initiates contact, and our IA concierge handles the rest, questions, availability, confirmation, reminders, and results notification. with human escalation available at any point for complex cases.

Muguerza Concierge does not replace the administrative team. It protects their capacity for the interactions that require judgment – insurer coordination, clinical escalation, exception handling – by absorbing the high-volume, low-complexity interactions that currently consume their time and degrade the patient experience.

Together, **Muguerza Connect** and **Muguerza Concierge** form a two-layer digital infrastructure: Connect manages the operational backend – workflows, pre-authorization, clinical coordination, and insurer tracking – while Concierge manages the relationship layer that patients and physicians experience.

In its initial deployment, **Muguerza Concierge** operates exclusively within the ambulatory platform. Over time, the same infrastructure can be extended to Muguerza's broader private physician network converting a competitive differentiator of the ambulatory model into a scalable digital service for the full Muguerza ecosystem.



5.5 Patient and physician journeys: Experience flowchart for our ambulatory model

The ambulatory experience is separated into 3 distinct modules: our WhatsApp communication channel, which is directly connected to Muguerza Connect, our own physician faced CRM, and our operation at the new fully integrated ambulatory Clinics.

The following link includes a detailed patient and physician experience flows, operational touchpoints, escalation paths, document intake appointment scheduling, reminders, results delivery, check-in, CRM workflows, and clinic variants for both existing hospitals and new ambulatory spokes. The goal is to show how the digital and physical components work together as one integrated ambulatory model, from first patient contact to clinical follow-up and operational execution.

Digital Layer Ambulatory Model – Journey and flowcharts for WhatsApp Concierge and Muguerza Connect

- To take an in-depth look of our WhatsApp journey for patients Click the “WhatsApp – Paciente” section
- To see our Muguerza Connect flows and understand the function of the platform Click “Connect-CRM” section
- To check out the mapped process for our clinics Click “Clínica operación” Section





6. TECHNICAL ANALYSIS & DIGITAL IMPLEMENTATION APPROACH

The ambulatory Proposal already established includes several digital innovations that will make this business model disruptive and difficult to replicate making Muguerza the leaders in ambulatory services at Northern Mexico, capturing and retaining not only patients but physicians making our digital layer indispensable for our stakeholders when searching for ambulatory Services.

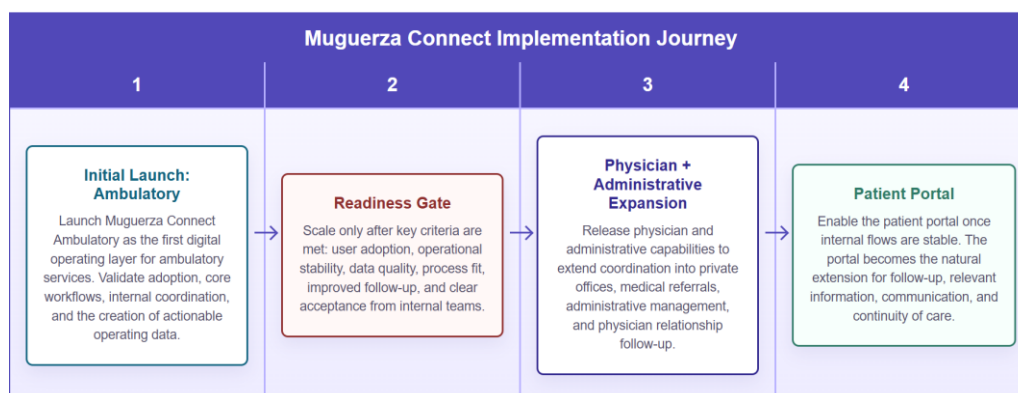
6.1 Digital Layer: Technical Features of Muguerza Connect

In this section we are doing an in-depth exploration of Muguerza Connect interface and features.

The solution is separated into 3 independent modules for our different stakeholders; the solution was developed for ambulatory services but escalated to private physician offices in Muguerza hospital and their administration team.

The current Ambulatory module of Muguerza Connect is in advanced beta and is planned to be the first launched as a key pillar of our ambulatory services model.

Muguerza Connect implementation journey



The Journey explores the implementation plan for the platform and establishes our expansion plan to introduce the physician, administrative and patient module of the platform.

The UX was designed to be easy to navigate and intuitive for daily use without the need for constant capacitation, the delivery model is simple, the IT team managing the platform will hand out specific credentials for our stakeholders, allowing them to access their specific portal molded to their needs.

Once the credential is delivered the clinic administrative team will be able to access the platform and explore the different features that the app offers.

Muguerza Connect ambulatory module Features

- **Dashboard:** A daily operational control panel for the ambulatory clinic team. It centralizes today's appointments, active escalations, check-ins, procedures in progress, pre-authorizations, and live resource occupancy so staff can understand the clinic's status at a glance.
- **Patients:** A searchable ambulatory patient registry with insurance, policy, visit history, current treatment status, upcoming appointments, and active alerts. Each patient profile opens into a



longitudinal view with appointments, pre-authorizations, results, conversations, resource assignments, and timeline history.

- **Calendar:** A full appointment management workspace for ambulatory services. Staff can create new visits, filter by status or service type, track payment and pre-authorization status, assign operational resources, and move patients through scheduled, check-in, in-progress, and completed states.
- **Pre auth:** A dedicated pre-authorization work queue for insurer-based services. It allows the clinic team to track requests by status, review insurer folios, identify pending approvals, and prevent services from moving forward when required authorization has not been cleared.
- **Inbox:** A CRM-style communication inbox connected to the patient journey. It organizes conversations by patient, channel, intent, unread status, and human handoff state, allowing the administrative team to follow up on appointments, pre-authorizations, results, escalations, and general requests.
- **Infrastructure:** A live capacity map for clinic resources such as infusion chairs, lab stations, imaging rooms, surgery rooms, and consult rooms. The module shows which resources are free, occupied, or over time, helping staff coordinate throughput and release capacity when services are completed.
- **Results:** A results tracking area for labs, imaging, and procedure notes. It highlights critical findings, separates notified from pending results, and links each result back to the patient record so follow-up does not depend on manual tracking.
- **Insurers:** An insurer performance summary that groups clinic activity by payor. It shows total appointments, completed services, pending pre-authorizations, and approved authorizations, giving the team visibility into insurer-related bottlenecks.
- **Performance:** A financial and operational analytics module for the ambulatory clinic. It tracks collected revenue, completed services, unique patients, estimated margin, insurer pipeline, pre-auth-related cancellations, monthly trends, payment models, and service-level performance.

In the following video there is a full user interface demo for a CEI's ambulatory clinic, with real case uses of the platform [MUGUERZA CONNECT DEMO VIDEO](#)





6.2 Digital Layer: Business Legal Requirements

Every new ambulatory clinic requires COFEPRIS authorization before opening. This institution is Mexico's equivalent of the U.S. FDA; These legal requirements are already well known and can be managed by the corresponding legal team. Nevertheless, there are several legal requirements that apply to the digital aspect of the proposal that Muguerza must consider.

- **Digital Platform – NOM-024 and SIRES Compliance (Muguerza Connect)**

As of January 2026, following a reform to the Ley General de Salud published in the DOF, digital management of clinical information became a legal obligation for all public and private health establishments in Mexico. [Doctoralia](#)

This makes Muguerza Connect NOM-024 compliance non-negotiable from day one.

- **Patient Data Privacy – LFPDPPP and WhatsApp Risk**

Health data is classified as *sensitive personal data* under the LFPDPPP – the category with the highest level of legal protection in Mexico. Sanctions for non-compliance can reach up to MXN \$18 million. In 2025, healthcare sector violations represented one of the largest accumulated fine amounts from INAI. [SaludTotal](#)

The business Development team objective is to expand Muguerza's reach beyond current capabilities, including new opportunities, and business models. This brief sub-section shows the legal implications that come with new disruptive models.

7. AMBULATORY PLATFORM: IMPLEMENTATION ROADMAP

This roadmap is intentionally conservative in the first year. It does not assume that Muguerza should immediately open multiple sites, launch several specialties, or replicate a full Oncare-style system from day one. Instead, it proposes the minimum viable platform required to prove the thesis: that Muguerza can use its hospital infrastructure, insurer relationships, physician network, and brand credibility to build a lower-cost, premium, digitally coordinated ambulatory model.

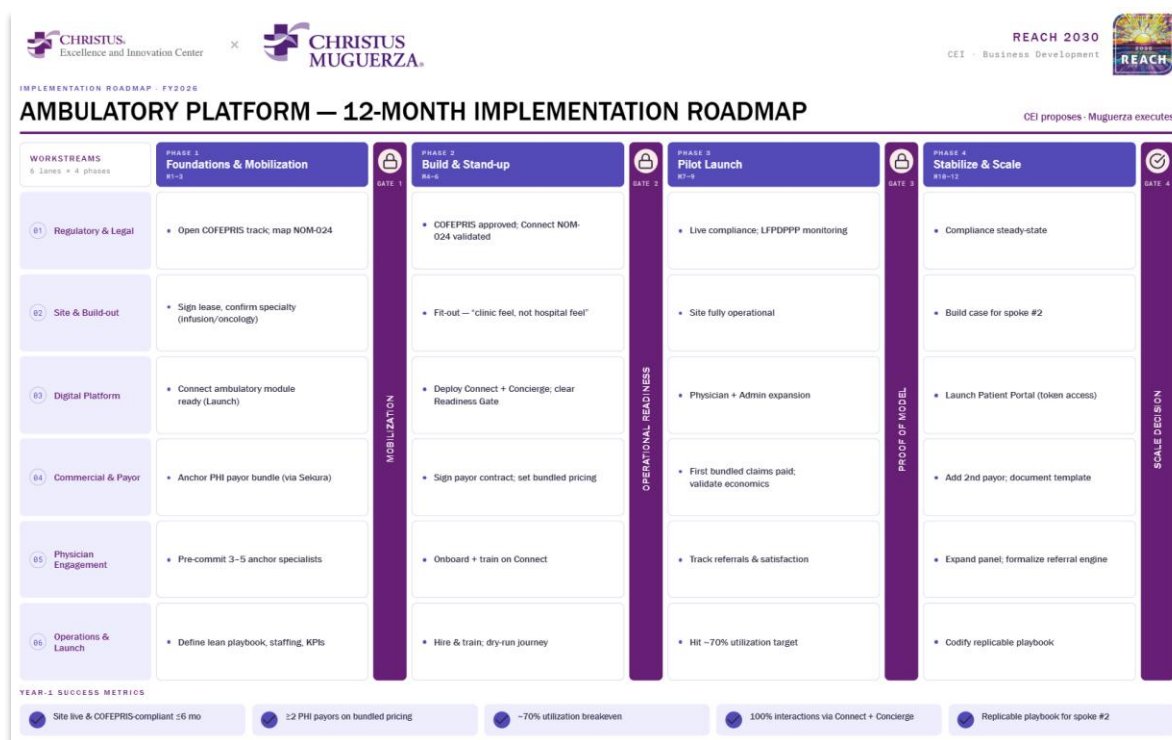
The first spoke is the proof point. If the pilot works, Muguerza does not only gain one clinic. It gains a replicable ambulatory operating system: a site template, a specialty playbook, a payor contracting model, a physician referral engine, and a digital backbone. That is the strategic asset CEI is proposing.

The pilot is designed around a hub-and-spoke logic. Hospital Muguerza Alta Especialidad functions as the clinical hub, escalation net, and brand anchor. The ambulatory clinic functions as the spoke: a premium, non-hospital-feeling environment located near the target AB/C+ insured population and operated under an asset-light lease model when possible. This allows Muguerza to reduce upfront CAPEX, accelerate time-to-market, and preserve flexibility if the site does not perform as expected.

The roadmap is organized into four quarterly phases and six parallel workstreams. Each phase closes with a decision gate. No phase should advance until the required operational, regulatory, digital, commercial, and clinical conditions are cleared.



Link to roadmap - [Ambulatory ROADMAP](#) (ZOOM OUT IN CHROM FOR A BETTER VIEW)



7.1 Road Map Logic

The 12-month sequence follows four principles:

First, the model must be proven before it is scaled. The first clinic should validate the operating model, not maximize the number of services offered from day one.

Second, the specialty must remain focused. A single specialty spoke allows Muguerza to standardize staffing, workflows, clinical protocols, supply needs, insurer pricing, and patient experience. This is what makes bundled pricing and predictable unit economics possible.

Third, the digital layer must be live before the clinic scales volume. Muguerza Connect and Muguerza Concierge are not optional enhancements. They are the operating infrastructure that prevents the new clinic from reproducing the same administrative friction seen in the current hospital experience.

Fourth, the payor relationship must be built into the model from the beginning. The ambulatory clinic only becomes strategically relevant if it gives PHI payors a lower-cost, easier-to-authorize, clinically safe alternative to hospital-based specialty care.

7.2 Core Workstreams

1. **Regulatory & Legal:** Responsible for COFEPRIS authorization, NOM-024/SIRES validation for Muguerza Connect, and LFPDPPP compliance for the handling of sensitive health data through Connect and WhatsApp Concierge.
2. **Site & Build-out:** Responsible for lease execution, architectural fit-out, equipment installation, clinic branding, patient experience design, and ensuring the facility feels like a premium ambulatory clinic rather than a hospital department.



3. Digital Platform: Responsible for deploying Muguerza Connect and Muguerza Concierge, including scheduling, patient registry, calendar, pre-authorization queue, inbox, infrastructure visibility, results, insurer tracking, and performance dashboards.
4. Commercial & Payor: Responsible for creating the first bundled-price episode with an anchor PHI payor, using existing Muguerza relationships and the Sekura broker channel as the entry point.
5. Physician Engagement: Responsible for identifying, pre-committing, onboarding, and training anchor specialists who will serve as the first referral engine for the pilot.
6. Operations & Launch: Responsible for staffing, training, dry-runs, patient journey validation, KPI design, utilization tracking, and operational playbook creation.

7.3 Roadmap Phases in detail

Phase – 1, Months 1-3: Foundations & Mobilization

The first phase is about defining the model before committing capital. During this stage, Muguerza must decide how the pilot will be deployed: either through the organic path inside an existing Muguerza facility or through the inorganic path in a fully leased external site. This decision is critical because it determines the regulatory route, investment level, build-out timeline, operating structure, and launch risk.

In this phase, Muguerza should confirm the pilot specialty, with infusion / oncology as the recommended starting point because it aligns with recurring outpatient demand, PHI payor interest, and the competitive lessons from Alivia and Oncare. The team must also open the COFEPRIS track, map NOM-024 requirements for Muguerza Connect, define LFPDPPP requirements for WhatsApp Concierge, and identify the first payor opportunity for bundled or preferred ambulatory pricing.

At the same time, Muguerza should pre-commit the first group of anchor specialists who will refer patients into the model. These physicians are not only clinical users; they are the first referral engine of the platform.

What must be done: select organic vs. inorganic path, confirm specialty, define the site, open regulatory track, define digital requirements, identify first payor opportunity, and pre-commit anchor physicians.

Gate 1 – Mobilization: deployment path selected, specialty confirmed, COFEPRIS track opened, first payor opportunity identified, and anchor physicians committed

Phase – 2, Months 4-6: Build & Stand-up

The second phase is about making the clinic operationally ready. Once the deployment path is selected, Muguerza must prepare the physical site, install the digital backbone, train the teams, and validate the complete patient journey before opening.

If the organic path is selected, the existing Muguerza space must be redesigned so it feels and operates like a dedicated ambulatory clinic, not like a hospital department. If the inorganic path is selected, the leased site must be adapted under an asset-light logic, with space, equipment, technology, and furniture aligned to the selected specialty.

This phase also includes deploying Muguerza Connect and Muguerza Concierge. Connect should manage scheduling, patient registry, pre-authorization tracking, insurer status, clinical documentation, results, and performance monitoring. Concierge should become the WhatsApp front door for patients



and physician offices, handling intake, reminders, document collection, status updates, and escalation when needed.

Commercially, this is the phase where the first payor pathway should move from concept to contract. Operationally, the staff must be hired and trained, and the full patient journey must be tested before go-live.

What must be done: complete site adaptation, finish regulatory requirements, deploy Connect and Concierge, sign the first payor pathway, onboard physicians, hire and train staff, and dry run the full patient journey.

Gate 2 – Operational Readiness: site ready, staff trained, Connect and Concierge deployed, first payor pathway signed or approved, and full journey tested end-to-end.

Phase – 3, Months 7-9: Pilot Launch

The third phase is the controlled launch with real patients. The objective is not to maximize volume immediately. The objective is to prove that the model works under real operating conditions.

During this phase, all patient, physician, administrative, and insurer interactions should run through Muguerza Connect and Concierge. The team must avoid creating a parallel manual process, because that would reproduce the same friction the ambulatory strategy is trying to eliminate.

Muguerza should monitor the real performance of the clinic: patient wait times, authorization turnaround, payor response, physician referral conversion, cancellations, utilization, revenue per resource, patient satisfaction, physician satisfaction, and claim behavior. This is also when the first bundled or preferred pathway claims should be processed to validate the commercial model.

The pilot must also confirm whether the clinic is moving toward the expected utilization discipline. The target should trend toward approximately 70% utilization, which is the breakeven discipline reference used in the operating model.

What must be done: launch with real patients, route interactions through Connect and Concierge, process first claims, measure patient and physician satisfaction, track utilization, and identify operational friction points.

Gate 3 – Proof of Model: first patients treated, claims processed, utilization trending toward target, physician referrals active, patient satisfaction measured, and early unit economics validated.

Phase – 4, Months 10-12: Stabilize & Scale

The fourth phase is about turning the pilot into a repeatable platform. By this stage, Muguerza should use real data from the pilot to decide whether to expand the same specialty, open a second site, add another payor, or prepare a higher-acuity ambulatory service line.

The team must document the operating playbook: staffing model, patient journey, physician referral workflow, insurer workflow, pre-authorization process, digital requirements, KPI dashboard, escalation criteria, and build-out template. This playbook is the strategic asset that allows Muguerza to scale without reinventing the model at every site.



Commercially, Muguerza should use pilot results to add a second PHI payor or prepare the second payor negotiation. Physician engagement should also expand beyond the anchor group into a broader referring panel.

What must be done: stabilize operations, document the playbook, refine Connect and Concierge, expand the physician panel, prepare a second payor, and build the business case for the next move.

Gate 4 – scale Decision: unit economics validated, playbook documented, second payor in progress, physician panel expanded, and board-ready recommendation prepared for site #2, specialty expansion, or both.

7.3 Mockup ambulatory clinic – Inorganic Expansion



Note: This Mock-Up was directly created from the business development opportunity that was found near Hospital Alta Especialidad in Alta Engracia ([imbuebles24](https://www.imbuebles24.com))





7.3 Year 1 Success Metrics

By the end of the first year, the pilot should be evaluated against a focused set of metrics: site live and COFEPRIS-compliant within six months; first payor pathway active by launch; second payor in progress by month 12; 3–5 anchor specialists actively referring; patient and physician interactions routed through Connect and Concierge; utilization trending toward approximately 70%; first claims processed; and a replicable playbook ready for the next site or specialty expansion.

The goal of Year 1 is not only to open a clinic. The goal is to prove that Muguerza can operate a separate, standardized, digitally enabled, payor-connected ambulatory model that can compete with specialized outpatient platforms and later scale across the market.



8. CONCLUSIONS: WHAT WE TAKE AWAY AND WHY IT IS MARKET-PROVEN

This proposal does not ask Muguerza to bet on a theory. Every component of the model is already operating – profitably and at scale – in real markets. The four business case studies lead to a single conclusion: the ambulatory opportunity is not an unproven idea; it is an assembly problem. The pieces exist and win independently; no operator in northern Mexico has yet combined them. Muguerza is the one player positioned to do so.

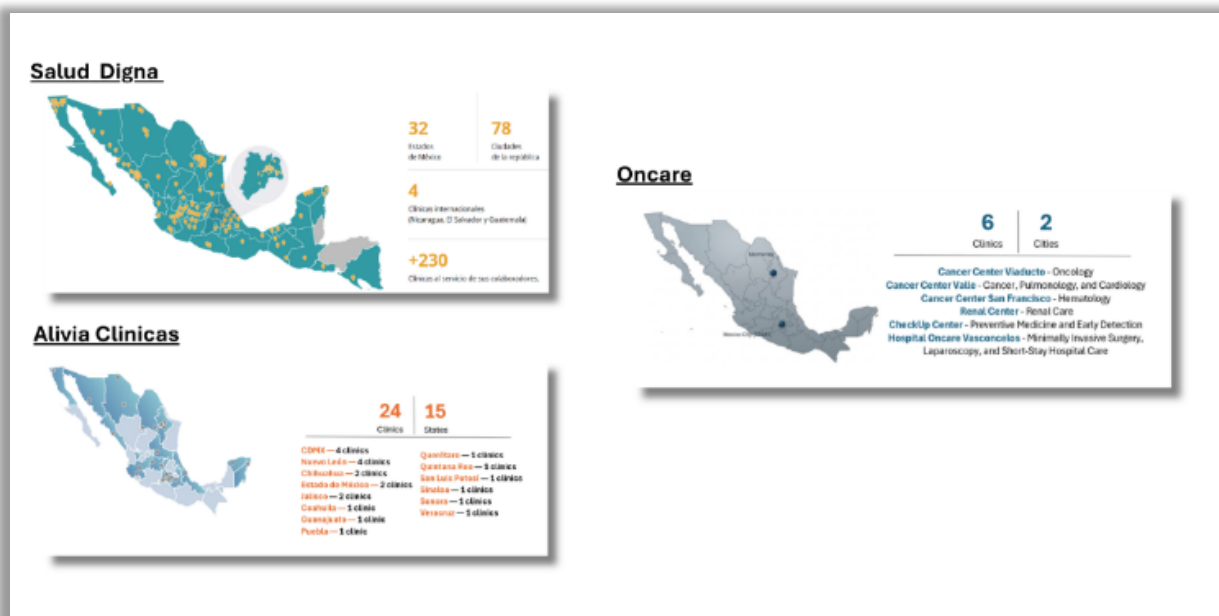
What CEI proposes Muguerza adopt, and the proof behind each piece:

- **The operating skeleton – USPI / Tenet:** A specialized, asset-light, physician-aligned platform captures the margin a full-service hospital cannot, by shedding the fixed-cost apparatus the procedure does not need. This is not projected: USPI has held a ~40% adjusted EBITDA margin across 2019–2025, through a pandemic and an inflation cycle, while scaling to 533 ASCs. The skeleton works at scale.
- **The patient experience – Alivia:** A deliberately non-hospital clinic environment paired with a WhatsApp-native journey strips administrative friction out of recurring care. Proven today in Monterrey, in the exact AB/C+ insured segment Muguerza targets.
- **The digital and physician layer – Oncare = Muguerza Connect:** A dual physician/patient platform turns independent specialists into a captive referral network and keeps patients inside the ecosystem. Proven in Monterrey by Oncare. **Muguerza Connect** is the owned version of this layer, already in advanced beta.
- **The operational discipline – Salud Digna:** Lean standardization, real-time operational dashboards, and asset-light leasing make pricing, scale, and speed of expansion possible – all routed through a WhatsApp-native front door that absorbs booking, payment, reminders, and results without front-desk dependency. This is the operating logic **Muguerza Concierge** replicates. Proven at national scale and studied by Harvard Business School.
- **The unfair advantage only Muguerza has – Brand, hospital adjacency, payor connectivity:** What took Alivia a decade to build, insurer relationships, clinical credibility, an escalation net, Muguerza already owns. This is the one ingredient every competitor had to build from zero, and the reason Muguerza can move faster and safer than any standalone operator.

Overall, the proposal stands out because it combines a margin structure proven at scale, an experience and digital layer proven locally, and operational discipline proven nationally – assembled on top of assets only Muguerza possesses. None of it is hypothetical. USPI is how the model makes money; Alivia, Oncare, and Salud Digna are how it looks, feels, and runs on the ground; Muguerza is the operator with the brand, the hospital, and the payors to execute it. The market has already validated every part of the model. The only thing missing is the operator that puts them together – and that is the opportunity CEI is recommending Muguerza capture.

9. APPENDIX

The big three geographical footprint



Muguerza Connect Figures

